

Important: Please note only data that has clinical mapping will be shared/transmitted.

This health record is for date range : 05/01/2025 to 05/01/2026

Please note that below sections represents the most current data on record and are not specific to the date range :Demographics, Social History, Problems, Medications, Allergies, Immunizations, Health Concerns, and Medical Equipment (UDI).

Patient Details

Patient name	Contact info	Patient IDs
	2099 N WATERBROOK PL STAR, ID 83669-6127, US	RCSL7Nj9Sd-TwYvK3J0F4x0CI2SIVHFTdyo-kR7XI3I28340
November 3, 1988	tel:+1(310)-750-5113	Sex Female

LANGUAGE

Information not available

RACE

ETHNICITY

Care Giver	Kirk Paolinelli, FNP
Contact Info	1526 W USTICK RD STE 110 MERIDIAN, ID 83646-7741, US Tel: +1(208)-247-0142
Care Giver	Dina Mattingly, NP
Contact Info	1526 W USTICK RD STE 110 MERIDIAN, ID 83646-7741, US Tel: +1(208)-247-0142

Allergies

No Known Allergies

Results

Component	Value	Reference Range	Notes
Urine Drug Screen (In House) Order date: 10/20/2025 Reviewed date:10/20/2025 12:40:02 PM Interpretation: Performing Lab: Notes/Report:			
AMP	NEG		
OPI	NEG		
MET	NEG		
BZO	NEG		
COC	NEG		

MTD	NEG		
OXY	NEG		
BUP	NEG		
MDMA	NEG		
THC	NEG		
PCP	NEG		
BAR	NEG		

Reason For Referral

Referral Date	04/27/2026
Referral Status	Open
Reason	Comprehensive evaluation for ASD and other potential diagnoses
Diagnosis 1	Attention-deficit hyperactivity disorder, combined type (F90.2)
Diagnosis 2	Generalized anxiety disorder (F41.1)
Diagnosis 3	Other disorders of psychological development (F88)
Referral Organization	ADD Clinic Of Idaho
Referring Provider First Name	Dina
Referring Provider Last Name	Mattingly
Referring Provider Speciality	Nurse Practitioner
Referred Provider	Northwest Neurobehavioral
Referred Provider Specialty	Mental health team
Referral Priority	Routine

Addressed Referral details can be found under 'Consultation Request Notes' section

Medications

Medication	SIG (Take, Route, Frequency, Duration)	Notes	Start Date	End Date	Diagnosis (ICD Code)	Status
Lisdexamfetamine Dimesylate 40 MG Capsule	1 capsule in the morning Orally Once a day; Duration: 30 days		12/15/2025		Attention-deficit hyperactivity disorder, combined type (ICD_10 - F90.2)	Not-Taking
Lisdexamfetamine Dimesylate 40 MG Capsule	1 capsule in the morning Orally Once a day; Duration: 30 days		01/12/2026		Attention-deficit hyperactivity disorder, combined type (ICD_10 - F90.2)	Not-Taking
Methylphenidate HCl ER (OSM) 36 MG Tablet	1 tablet in the morning Orally Once a day; Duration: 30		02/23/2026		Attention-deficit hyperactivity disorder, combined type (ICD_10 -	Not-Taking

Extended Release	days				F90.2)	
Lisdexamfetamine Dimesylate 30 MG Capsule	1 capsule in the morning Orally Once a day; Duration: 30 days		03/23/2026		Attention-deficit hyperactivity disorder, combined type (ICD_10 - F90.2)	Active
Sertraline HCl 25 MG Tablet	1 tablet Orally Once a day; Duration: 30 days		03/23/2026		Current episode of major depressive disorder without prior episode, unspecified depression episode severity (ICD_10 - F32.9)	Active
Propranolol HCl 10 MG Tablet	1 tablet Orally daily; Duration: 30 days As needed		01/25/2026		Performance anxiety (ICD_10 - F41.8)	Active
Lisdexamfetamine Dimesylate 30 MG Capsule	1 capsule in the morning Orally Once a day; Duration: 30 days		10/20/2025		Attention-deficit hyperactivity disorder, combined type (ICD_10 - F90.2)	Not-Taking
Mirtazapine 7.5 MG Tablet	1 tablet at bedtime Orally Once a day				Current episode of major depressive disorder without prior episode, unspecified depression episode severity (ICD_10 - F32.9)	Not-Taking
guanFACINE HCl ER 1 MG Tablet Extended Release 24 Hour	1 tablet Orally nightly; Duration: 90 days		12/15/2025		Attention-deficit hyperactivity disorder, combined type (ICD_10 - F90.2)	Not-Taking

Immunizations

No Information

Social History

Sex Observation

Social History Observation	Description
Sex Observation	Female

Social History

Section Notes:

Work as a director of project management of mountain marketing platform

Single 2 dogs no children

Problems

Problem Type	SNOMED Code	ICD Code	Dates	Problem Status	W/U Status	Risk	Notes

Problem	Generalized anxiety disorder (21897009)	Generalized anxiety disorder (F41.1)	Added On:03/23/2026	Active	confirmed		
Problem	Disorder of psychological development (192562009)	Other disorders of psychological development (F88)	Added On:04/27/2026	Active	confirmed		
Problem	Attention deficit hyperactivity disorder, combined type (31177006)	Attention-deficit hyperactivity disorder, combined type (F90.2)	Added On:10/20/2025	Active	confirmed		
Problem	Attention deficit hyperactivity disorder, predominantly inattentive type (disorder) (35253001)	Attention and concentration deficit (R41.840)	Added On:10/20/2025	Active	confirmed		
Problem	Attention deficit hyperactivity disorder (406506008)	ADHD (attention deficit hyperactivity disorder), combined type (F90.2)	Added On:09/24/2025	Active	confirmed		
Problem	Difficulty sleeping (301345002)	Sleep difficulties (G47.9)	Added On:10/20/2025	Active	confirmed		
Problem	Raynaud's disease (195295006)	Raynaud's phenomenon without gangrene (I73.00)	Added On:12/15/2025	Active	confirmed		
Problem	Major depression, single episode (36923009)	Current episode of major depressive disorder without prior episode, unspecified depression episode severity (F32.9)	Added On:09/24/2025	Active	confirmed		
Problem	Performance anxiety (279622009)	Performance anxiety (F41.8)	Added On:10/20/2025	Active	confirmed		

Vital Signs

Vital Sign	Value	Notes	Appt Date
Heart Rate	60 /min		04/27/2026
Respiratory Rate	18 /min		11/17/2025
Height-cm	157.48 cm		04/27/2026
Oximetry	98 %		04/27/2026
Blood pressure diastolic	70 mm Hg		04/27/2026
Weight-kg	65.23 kg		04/27/2026

Height	62 in		04/27/2026
Blood pressure systolic	110 mm Hg		04/27/2026
Weight	143.8 lbs		04/27/2026
BMI	26.3 kg/m2		04/27/2026

Procedures

Procedure	Date Ordered	Date Performed	Result	Body Site
Continuous Performance Task	09/24/2025	10/20/2025	N/A	

Encounters

Date	Time	Type	Facility	Location	Provider	Diagnosis
09/24/2025	03:15 PM	Office Visit, New Pt., Level 5 (99205)	ADD Clinic Of Idaho	1526 W USTICK RD STE 110 MERIDIAN, ID 83646-7741	Kirk Paolinelli	ADHD (attention deficit hyperactivity disorder), combined type F90.2 and Current episode of major depressive disorder without prior episode, unspecified depression episode severity F32.9
10/20/2025	12:30 PM	Office Visit, Est Pt., Level 5 (99215)	ADD Clinic Of Idaho	1526 W USTICK RD STE 110 MERIDIAN, ID 83646-7741	Dina Mattingly	Performance anxiety F41.8 ; Attention-deficit hyperactivity disorder, combined type F90.2 and Sleep difficulties G47.9
11/17/2025	10:30 AM	Office Visit, Est Pt., Level 5 (99215)	ADD Clinic Of Idaho	1526 W USTICK RD STE 110 MERIDIAN, ID 83646-7741	Dina Mattingly	Performance anxiety F41.8 ; Attention-deficit hyperactivity disorder, combined type F90.2 and Sleep difficulties G47.9
12/15/2025	03:00 PM	Office Visit, Est Pt., Level 5 (99215)	ADD Clinic Of Idaho	1526 W USTICK RD STE 110 MERIDIAN, ID 83646-7741	Dina Mattingly	Performance anxiety F41.8 ; Attention-deficit hyperactivity disorder, combined type F90.2 ; Sleep difficulties G47.9 ; Raynaud's phenomenon without gangrene I73.00 and Work stress Z56.6
01/26/2026	09:00 AM	Office Visit, Est Pt., Level 4 (99214)	ADD Clinic Of Idaho	1526 W USTICK RD STE 110 MERIDIAN, ID 83646-7741	Dina Mattingly	Attention-deficit hyperactivity disorder, combined type F90.2 ; Performance anxiety F41.8 ; Sleep difficulties G47.9 ; Work stress Z56.6 and Raynaud's phenomenon without gangrene I73.00
02/23/2026	10:00 AM	Office Visit, Est Pt., Level 4 (99214)	ADD Clinic Of Idaho	1526 W USTICK RD STE 110 MERIDIAN, ID 83646-7741	Dina Mattingly	Attention-deficit hyperactivity disorder, combined type F90.2 ; Current episode of major depressive disorder without prior episode, unspecified depression episode severity F32.9 ; Burn-out Z73.0 and Stress, not elsewhere classified Z73.3
03/23/2026	02:30 PM	Office Visit, Est Pt., Level 4 (99214)	ADD Clinic Of Idaho	1526 W USTICK RD STE 110 MERIDIAN, ID 83646-7741	Dina Mattingly	Attention-deficit hyperactivity disorder, combined type F90.2 ; Generalized anxiety disorder F41.1 and Current episode of major depressive disorder without prior episode,

						unspecified depression episode severity F32.9
04/27/2026	09:50 AM	Office Visit, Est Pt., Level 5 (99215)	ADD Clinic Of Idaho	1526 W USTICK RD STE 110 MERIDIAN, ID 83646-7741	Dina Mattingly	Attention-deficit hyperactivity disorder, combined type F90.2 ; Generalized anxiety disorder F41.1 and Other disorders of psychological development F88
01/09/2026	04:14 PM	Web Encounter	ADD Clinic Of Idaho	1526 W USTICK RD STE 110 MERIDIAN, ID 83646-7741	Dina Mattingly	
02/17/2026	04:08 PM	Web Encounter	ADD Clinic Of Idaho	1526 W USTICK RD STE 110 MERIDIAN, ID 83646-7741	Dina Mattingly	
02/18/2026	03:29 PM	Web Encounter	ADD Clinic Of Idaho	1526 W USTICK RD STE 110 MERIDIAN, ID 83646-7741	Dina Mattingly	

Medical Equipment

No Information

Assessments

Encounter Date	Diagnosis (ICD Code)	Treatment Notes	Section Notes
09/24/2025	ADHD (attention deficit hyperactivity disorder), combined type (ICD-10 - F90.2)		
09/24/2025	Current episode of major depressive disorder without prior episode, unspecified depression episode severity (ICD-10 - F32.9)		
		Based on the subjective hx, objective findings, screeners, as well as their Qb test results- pt does meet the DSM-5 criteria for ADHD-combined type. Symptoms are not otherwise explained by any physiological concerns or substance use. Discussed treatment options to manage ADHD including non-stimulant medication, stimulant medication, CBT. Discussed	

10/20/2025	Attention-deficit hyperactivity disorder, combined type (ICD-10 - F90.2)	risks and benefits of each treatment option. Pt verbalized understanding. Using shared decision making- Will start Vyvanse at 30 mg once daily for ADHD mgmt. Urine drug screen utilized- was NEG. IDPMP reviewed when considering controlled substance medication. VS reviewed and are WNL. ADD Controlled substance agreement form signed. Informed patient to take medication as prescribed. Take the stimulant in the morning. I educated the patient on the risk of potential abuse while taking stimulant, not to share the stimulant with anyone else, keep it locked and safe, do not break or chew the capsule unless advised otherwise. Stop taking the stimulant during active palpitations, chest pain, shortness of breath or any other cardiac symptom and seek immediate medical attention. Discussed risks vs benefits of treatment with medications today as well as possible adverse effects. Patient verbalized understanding today. Will follow-up in one month to evaluate medication effectiveness and tolerability or sooner if new, worsening, or persisting symptoms present.	
11/17/2025	Performance anxiety (ICD-10 - F41.8)	- Patient has propranolol available as needed but has not yet utilized it during stressful situations including recent podcast appearance- Anticipatory anxiety remains significant during performance scenarios- Plan: Continue propranolol as needed for performance anxiety and high-stress situations, trial full dose when appropriate stressful situation arises particularly during competitions or similar performance scenarios Discussed risks vs benefits of treatment with medications today as well as possible adverse effects. Patient verbalized understanding today.	
12/15/2025	Performance anxiety (ICD-10 - F41.8)	- Patient has propranolol available as needed but has not yet utilized it during stressful situations including recent podcast appearance- Anticipatory anxiety remains significant during performance scenarios- Plan: Continue propranolol as needed for performance anxiety and high-stress situations, trial full dose when appropriate stressful situation arises particularly during competitions or similar performance scenarios. This medication can also exacerbate raynaud symptoms, so utilize this as needed medication with caution. Discussed risks vs benefits of treatment with medications today as well as	

		possible adverse effects. Patient verbalized understanding today.	
01/26/2026	Attention-deficit hyperactivity disorder, combined type (ICD-10 - F90.2)	- Patient discontinued guanfacine after experiencing paradoxical effects including severe insomnia, increased irritability, and worsened mental chatter- Currently on Vyvanse 40 mg with mixed response; reports good coverage but feels dose may be slightly too high due to increased hyperfocus and recent elevation in resting heart rate- Patient describes feeling between 30-40 mg dosing, with 30 mg providing good effect but insufficient duration and 40 mg providing adequate coverage but potentially excessive intensity- Plan: Discontinue Vyvanse 40 mg, start Concerta 36 mg daily, Discussed strategies to help wit ADHD including: implement Pomodoro Technique for hyperfocus management with 25-30 minutes focused work followed by 5-minute breaksIDPMP reviewed when considering controlled substance medication. VS reviewed and are WNL. ADD Controlled substance agreement form signed. Informed patient to take medication as prescribed. Take the stimulant in the morning. Stop taking the stimulant during active palpitations, chest pain, shortness of breath or any other cardiac symptom and seek immediate medical attention. Discussed risks vs benefits of treatment with medications today as well as possible adverse effects. Patient verbalized understanding today. Possible Autism Spectrum Disorder:- Patient reports longstanding difficulties with social cues and communication, including literal thinking, difficulty with sarcasm recognition, and history of being misunderstood interpersonally- Describes significant masking behaviors and burnout from effort required to function socially- Reports hyperfixation patterns and has received feedback requiring self-coaching on interpersonal communication- Plan: Referral to GPS Psychology and Anxiety Clinic for comprehensive psychological evaluation specializing in adult female autism assessment Will follow-up in 4 weeks to evaluate medication effectiveness and tolerability of the Concerta or sooner if new, worsening, or persisting symptoms present.	
		- Patient has propranolol available as needed but has not yet utilized it during stressful situations including recent podcast	

01/26/2026	Performance anxiety (ICD-10 - F41.8)	appearance- Anticipatory anxiety remains significant during performance scenarios- Plan: Continue propranolol as needed for performance anxiety and high-stress situations, trial full dose when appropriate stressful situation arises particularly during competitions or similar performance scenarios. This medication can also exacerbate raynaud symptoms, so utilize this as needed medication with caution. Discussed risks vs benefits of treatment with medications today as well as possible adverse effects. Patient verbalized understanding today.	
02/23/2026	Attention-deficit hyperactivity disorder, combined type (ICD-10 - F90.2)		1. Attention-Deficit/Hyperactivity Disorder, Combined Type - Currently managed on Concerta with good symptom control and improved executive functioning. Patient reports effective coverage and ability to maintain focus while preserving cognitive flexibility. 2. Depressive Disorder - Patient presents with flat affect, anhedonia, low energy, and sense of life passing by in context of chronic stress adaptation over more than 10 years. Symptoms distinct from major depressive disorder, characterized by emotional flatness and survival mode rather than typical depressive cognitions. Previous positive response to SSRI (Lexapro 5mg) supports pharmacologic intervention. 3. Burnout - Chronic occupational stress with adaptation of nervous system to prolonged fight-or-flight state. Patient describes baseline as

			survival mode with loss of ability to experience joy or stress response despite ongoing stressors.
02/23/2026	Current episode of major depressive disorder without prior episode, unspecified depression episode severity (ICD-10 - F32.9)		1. Attention-Deficit/Hyperactivity Disorder, Combined Type - Currently managed on Concerta with good symptom control and improved executive functioning. Patient reports effective coverage and ability to maintain focus while preserving cognitive flexibility. 2. Depressive Disorder - Patient presents with flat affect, anhedonia, low energy, and sense of life passing by in context of chronic stress adaptation over more than 10 years. Symptoms distinct from major depressive disorder, characterized by emotional flatness and survival mode rather than typical depressive cognitions. Previous positive response to SSRI (Lexapro 5mg) supports pharmacologic intervention. 3. Burnout - Chronic occupational stress with adaptation of nervous system to prolonged fight-or-flight state. Patient describes baseline as survival mode with loss of ability to experience joy or stress response despite ongoing stressors.
			1. Generalized Anxiety Disorder - Responding well to sertraline with significant early

03/23/2026	Generalized anxiety disorder (ICD-10 - F41.1)		improvement in anxiety symptoms, decreased reactivity, and improved stress management. Reports 20-25% improvement within first 2-3 weeks, indicating good medication match. 2. Attention-Deficit/Hyperactivity Disorder, combined type - Current stimulant (Concerta) providing inadequate symptom control since reintroduction. Likely either overmedicated in context of improved anxiety baseline, undermedicated, or on suboptimal medication. Previously responded well to Vyvanse prior to anxiety treatment. 3. Autism Spectrum Disorder (rule out)- Under consideration, referral placed for comprehensive evaluation.
03/23/2026	Attention-deficit hyperactivity disorder, combined type (ICD-10 - F90.2)		1. Generalized Anxiety Disorder - Responding well to sertraline with significant early improvement in anxiety symptoms, decreased reactivity, and improved stress management. Reports 20-25% improvement within first 2-3 weeks, indicating good medication match. 2. Attention-Deficit/Hyperactivity Disorder, combined type - Current stimulant (Concerta) providing inadequate symptom control since reintroduction. Likely either overmedicated in context of improved anxiety

			baseline, undermedicated, or on suboptimal medication. Previously responded well to Vyvanse prior to anxiety treatment. 3. Autism Spectrum Disorder (rule out)- Under consideration, referral placed for comprehensive evaluation.
10/20/2025	Performance anxiety (ICD-10 - F41.8)	- Patient experiences significant performance anxiety during competitive dog sports with somatic symptoms including nausea and diarrhea- Anxiety appears situational and performance-specific rather than generalized- Plan: Propranolol 10 mg as needed taken at least one hour before performance events, can cut tablet in half initially to assess tolerance, trial on non-performance day first to assess tolerance, discontinue and seek immediate medical attention if experiencing shortness of breath or chest tightness, syncope, presyncope. Discussed risks vs benefits of treatment with medications today as well as possible adverse effects. Patient verbalized understanding today.	
04/27/2026	Generalized anxiety disorder (ICD-10 - F41.1)		1. Generalized Anxiety Disorder - Excellent response to sertraline with resolution of chronic excessive worry, overplanning, fight-or-flight baseline, and anticipatory anxiety. Patient now recognizes previous anxiety symptoms that were not identified as pathological. 2. Attention-Deficit/Hyperactivity Disorder, predominantly inattentive type - Symptoms of inattention, impulsivity, and executive dysfunction persist but are more manageable with anxiety treatment. Focus difficulties and task completion challenges remain present though improved from baseline. 3. Autism Spectrum Disorder (provisional, evaluation pending)

			- Clinical presentation consistent with ASD including social communication difficulties, concrete and literal thinking, eye contact difficulties, masking behaviors, childhood special interests, and challenges with social reciprocity. Comprehensive neuropsychological evaluation recommended for diagnostic clarification.
04/27/2026	Attention-deficit hyperactivity disorder, combined type (ICD-10 - F90.2)		1. Generalized Anxiety Disorder - Excellent response to sertraline with resolution of chronic excessive worry, overplanning, fight-or-flight baseline, and anticipatory anxiety. Patient now recognizes previous anxiety symptoms that were not identified as pathological. 2. Attention-Deficit/Hyperactivity Disorder, predominantly inattentive type - Symptoms of inattention, impulsivity, and executive dysfunction persist but are more manageable with anxiety treatment. Focus difficulties and task completion challenges remain present though improved from baseline. 3. Autism Spectrum Disorder (provisional, evaluation pending) - Clinical presentation consistent with ASD including social communication difficulties, concrete and literal thinking, eye contact difficulties, masking behaviors, childhood special

			interests, and challenges with social reciprocity. Comprehensive neuropsychological evaluation recommended for diagnostic clarification.
04/27/2026	Other disorders of psychological development (ICD-10 - F88)		1. Generalized Anxiety Disorder - Excellent response to sertraline with resolution of chronic excessive worry, overplanning, fight-or-flight baseline, and anticipatory anxiety. Patient now recognizes previous anxiety symptoms that were not identified as pathological. 2. Attention-Deficit/Hyperactivity Disorder, predominantly inattentive type - Symptoms of inattention, impulsivity, and executive dysfunction persist but are more manageable with anxiety treatment. Focus difficulties and task completion challenges remain present though improved from baseline. 3. Autism Spectrum Disorder (provisional, evaluation pending) - Clinical presentation consistent with ASD including social communication difficulties, concrete and literal thinking, eye contact difficulties, masking behaviors, childhood special interests, and challenges with social reciprocity. Comprehensive neuropsychological evaluation recommended for diagnostic clarification.

11/17/2025	Attention-deficit hyperactivity disorder, combined type (ICD-10 - F90.2)	- Patient reports significant improvement in executive functioning and quality of life on Vyvanse 30 mg over the past month- Medication duration has shortened from initial 10-12 hours to approximately 6-7 hours with effects ending around 3 PM by week 3-4- Return of meal management difficulties, hyper-focus issues, and challenges with laundry, dishes, and grocery shopping as medication wears off- Patient tolerates medication well with only mild dry mouth and no significant cardiovascular effects- Plan: Increase Vyvanse from 30 mg to 40 mg daily to optimize symptom coverage and extend duration of therapeutic effect IDPMP reviewed when considering controlled substance medication. VS reviewed and are WNL. ADD Controlled substance agreement form signed. Informed patient to take medication as prescribed. Take the stimulant in the morning. Stop taking the stimulant during active palpitations, chest pain, shortness of breath or any other cardiac symptom and seek immediate medical attention. Discussed risks vs benefits of treatment with medications today as well as possible adverse effects. Patient verbalized understanding today. Will follow-up in one month to evaluate medication effectiveness and tolerability or sooner if new, worsening, or persisting symptoms present.	
03/23/2026	Current episode of major depressive disorder without prior episode, unspecified depression episode severity (ICD-10 - F32.9)		1. Generalized Anxiety Disorder - Responding well to sertraline with significant early improvement in anxiety symptoms, decreased reactivity, and improved stress management. Reports 20-25% improvement within first 2-3 weeks, indicating good medication match. 2. Attention-Deficit/Hyperactivity Disorder, combined type - Current stimulant (Concerta) providing inadequate symptom control since reintroduction. Likely either overmedicated in

			context of improved anxiety baseline, undermedicated, or on suboptimal medication. Previously responded well to Vyvanse prior to anxiety treatment. 3. Autism Spectrum Disorder (rule out)- Under consideration, referral placed for comprehensive evaluation.
02/23/2026	Burn-out (ICD-10 - Z73.0)		1. Attention-Deficit/Hyperactivity Disorder, Combined Type - Currently managed on Concerta with good symptom control and improved executive functioning. Patient reports effective coverage and ability to maintain focus while preserving cognitive flexibility. 2. Depressive Disorder - Patient presents with flat affect, anhedonia, low energy, and sense of life passing by in context of chronic stress adaptation over more than 10 years. Symptoms distinct from major depressive disorder, characterized by emotional flatness and survival mode rather than typical depressive cognitions. Previous positive response to SSRI (Lexapro 5mg) supports pharmacologic intervention. 3. Burnout - Chronic occupational stress with adaptation of nervous system to prolonged fight-or-flight state. Patient describes baseline as survival mode with loss of ability

			to experience joy or stress response despite ongoing stressors.
01/26/2026	Sleep difficulties (ICD-10 - G47.9)	- Plan: Continue to monitor sleep during our f/u appts. Implement good sleep hygiene measures such as: - Decreasing stimuli 1 hr prior to sleep -Reserving room for sleep only -Continue to monitor sleep quality Avoid excessive caffeine or sugary drinks (ie: energy drinks/coffee), especially during the late afternoon/evening hours. Avoid excessive alcohol use. Continue to participate in some type of physical activity. Examples (gym, walking block, walking dog, swimming, or biking) Create a predictable and quiet night time routine. Example (hot shower, same bedtime, decreased tv, phone or computer use, cool room, dark room/black out curtains, sound machine). Stick to a regular sleep-walk schedule including weekends.	
12/15/2025	Attention-deficit hyperactivity disorder, combined type (ICD-10 - F90.2)	- Patient has been on Vyvanse 40 mg for approximately four weeks with some positive effects including improved confidence in tackling overwhelming tasks and resolution of dry mouth- Assessment of therapeutic response complicated by extreme work stress involving problematic employee situation- Patient reports difficulty determining if symptoms are medication-related versus stress-related- Continues to experience chronic overstimulation, edginess, and hypervigilance that predates current medication trial and appears related to longstanding unmanaged ADHD symptoms requiring excessive mental effort leading to burnout- Plan: Continue Vyvanse 40 mg daily, add guanfacine for combination therapy to provide broader symptom coverage and emotional regulation, start guanfacine at lowest dose, medication takes 2-3 weeks to show therapeutic effect and will assess over 6-8 weeks IDPMP reviewed when considering controlled substance medication. VS reviewed and are WNL. ADD Controlled substance agreement form signed. Informed patient to take	

		medication as prescribed. Take the stimulant in the morning. Stop taking the stimulant during active palpitations, chest pain, shortness of breath or any other cardiac symptom and seek immediate medical attention. Discussed risks vs benefits of treatment with medications today as well as possible adverse effects. Patient verbalized understanding today. Will follow-up in 6 weeks to evaluate medication effectiveness and tolerability of the Guanfacine or sooner if new, worsening, or persisting symptoms present.	
10/20/2025	Sleep difficulties (ICD-10 - G47.9)	- Patient reports poor sleep with delayed sleep onset consistent with ADHD-related restlessness and difficulty unwinding- Sleep issues likely multifactorial with significant component related to unmanaged ADHD causing mental and physical exhaustion manifesting as nighttime restlessness and racing thoughts- Plan: Continue mirtazapine 7.5 mg until ADHD optimally managed, plan to taper mirtazapine once ADHD treatment established, consider guanfacine as ADHD-specific sleep aid if needed after mirtazapine discontinuation, magnesium glycinate 200-400 mg at bedtime may help with sleep and restlessness	
12/15/2025	Sleep difficulties (ICD-10 - G47.9)	- Patient discontinued mirtazapine by tapering and was sleeping well until current work stress began- Now experiencing racing thoughts and rumination at bedtime which is uncharacteristic- Symptoms appear directly related to heightened stress level from work situation- Plan: Guanfacine expected to help with racing thoughts and restless sleep when taken a couple hours before bedtime, continue monitoring sleep patterns as work stress resolves Implement good sleep hygiene measures such as: - Decreasing stimuli 1 hr prior to sleep -Reserving room for sleep only -Continue to monitor sleep quality Avoid excessive caffeine or sugary drinks (ie: energy drinks/coffee), especially during the late afternoon/evening hours. Avoid excessive alcohol use. Continue to participate in some type of physical activity. Examples (gym, walking block, walking dog, swimming, or biking) Create a predictable and quiet night time routine. Example (hot shower, same bedtime, decreased tv, phone or computer use, cool room, dark room/black out curtains, sound machine).	

		Stick to a regular sleep-walk schedule including weekends.	
01/26/2026	Work stress (ICD-10 - Z56.6)	- Patient experiencing ongoing work-related stress and difficulty with career decision-making- Reports hyperfocus consuming significant energy at work, leaving little capacity for personal projects- Struggling with trusting decision-making regarding career goals including side business development and book writing project- Plan: Referral for counseling/therapy with weekly, bi-weekly, or monthly sessions focusing on ADHD-related challenges and career concerns - Continue monitoring as work situation resolves	
02/23/2026	Stress, not elsewhere classified (ICD-10 - Z73.3)		1. Attention-Deficit/Hyperactivity Disorder, Combined Type - Currently managed on Concerta with good symptom control and improved executive functioning. Patient reports effective coverage and ability to maintain focus while preserving cognitive flexibility. 2. Depressive Disorder - Patient presents with flat affect, anhedonia, low energy, and sense of life passing by in context of chronic stress adaptation over more than 10 years. Symptoms distinct from major depressive disorder, characterized by emotional flatness and survival mode rather than typical depressive cognitions. Previous positive response to SSRI (Lexapro 5mg) supports pharmacologic intervention. 3. Burnout - Chronic occupational stress with adaptation of nervous system to prolonged fight-or-flight state. Patient describes baseline as

			survival mode with loss of ability to experience joy or stress response despite ongoing stressors.
11/17/2025	Sleep difficulties (ICD-10 - G47.9)	Patient experienced minimal sleep difficulties with only 2 nights of delayed sleep onset during week 1 of current Vyvanse dose- Plan: Monitor for sleep disturbances with dose increase, gradual discontinuation of mirtazapine by continuing current regimen of half pill every other night Implement good sleep hygiene measures such as: - Decreasing stimuli 1 hr prior to sleep -Reserving room for sleep only -Continue to monitor sleep quality Avoid excessive caffeine or sugary drinks (ie: energy drinks/coffee), especially during the late afternoon/evening hours. Avoid excessive alcohol use. Continue to participate in some type of physical activity. Examples (gym, walking block, walking dog, swimming, or biking) Create a predictable and quiet night time routine. Example (hot shower, same bedtime, decreased tv, phone or computer use, cool room, dark room/black out curtains, sound machine). Stick to a regular sleep-walk schedule including weekends.	
12/15/2025	Raynaud's phenomenon without gangrene (ICD-10 - I73.00)	- Patient reports new onset blue discoloration of lips and fingers particularly in morning that improves throughout the day with movement and warming- Symptoms are currently tolerable and resolve by afternoon -Denies ulcers, skin breakdown, severe pain, asymmetric symptoms- Plan:Discussed stimulant-related vasoconstriction as a known adverse effect. Reviewed conservative measures and risks/benefits of continuing stimulant therapy. Plan to reduce dose/switch medication if symptoms persist or worsen. Patient advised to report any pain, ulcers, or worsening discoloration immediately. Consider switching to methylphenidate class stimulants if symptoms become intolerable or symptoms worsen. Counsel on: Keeping hands/feet warm	

		Avoiding cold exposure Hydration Avoiding nicotine and decongestants Monitor closely	
01/26/2026	Raynaud's phenomenon without gangrene (ICD-10 - I73.00)	- Patient reports new onset blue discoloration of lips and fingers particularly in morning that improves throughout the day with movement and warming- Symptoms are currently tolerable and resolve by afternoon -Denies ulcers, skin breakdown, severe pain, asymmetric symptoms- Plan:Discussed stimulant-related vasoconstriction as a known adverse effect. Reviewed conservative measures and risks/benefits of continuing stimulant therapy. Plan to reduce dose/switch medication if symptoms persist or worsen. Patient advised to report any pain, ulcers, or worsening discoloration immediately. Switching to methylphenidate class stimulant (Concerta)- may have potential to lessen symptoms Keeping hands/feet warm Avoiding cold exposure Hydration Avoiding nicotine and decongestants Monitor closely	
12/15/2025	Work stress (ICD-10 - Z56.6)	Work-Related Stress:- Extreme work stress involving problematic employee situation that escalated to C-level management over past four weeks- Stress contributing to difficulty assessing medication effectiveness and sleep disturbance- Plan: Continue monitoring as work situation resolves	
		Psychopharmacology: - Start sertraline 12.5mg daily for 1 week, then increase to 25mg daily for depression and chronic stress-related symptoms. Patient to cut 25mg tablet in half for initial week to minimize activation and side effects given sensitivity to medications. If activation occurs after increasing to 25mg, patient instructed to return to 12.5mg for another week before re-attempting increase. Anticipate therapeutic dose will be 25-50mg given previous response to low-dose Lexapro. - Continue Concerta at 36 mg once daily for ADHD management. Patient reports good coverage and effectiveness.	

02/23/2026	Other	Will reassess dose in 4-5 weeks. - Patient to take sertraline with food, paired with morning Concerta dose, to minimize GI upset. Referrals: - ASD evaluation referral placed to GPS Psychology and Anxiety Clinic after initial referral site did not accept insurance. Patient completed release of information form. - Counseling referral to be adjusted to specify EMDR training for trauma-focused therapy addressing chronic stress and hyper-independence patterns. Will ensure therapist has training in both cognitive behavioral therapy and EMDR. Patient may also benefit from brain spotting if meets criteria. Education: - Reviewed sertraline expected benefits including improvement in flat affect, anhedonia, and stress-related symptoms. Typical response time 4-6 weeks with possible initial activation in first 2 weeks, mitigated by slow titration. - Discussed common side effects: GI upset (nausea, diarrhea, constipation, stomach pain) particularly in first weeks, decreased libido, weight changes. - Reviewed black box warning for worsening depression, suicidal ideation with plan or intent - instructed to discontinue medication and contact provider immediately if occurs, call 911 if actively suicidal. - Discussed activation symptoms to monitor: increased anxiety, irritability, agitation, restlessness. If intolerable, reduce dose and contact provider. - Educated on chronic stress impact on nervous system and brain chemistry adaptation, validating patient's research and understanding of condition. - Encouraged continued exercise and outdoor activities as tolerated for mood and ADHD management. - Discussed importance of pursuing counseling, particularly EMDR, as critical component of treatment plan alongside medication management. Follow-Up: - Follow up in 1 month for medication monitoring and ADHD reassessment. - Patient instructed to contact provider sooner if intolerable side	1. Attention-Deficit/Hyperactivity Disorder, Combined Type - Currently managed on Concerta with good symptom control and improved executive functioning. Patient reports effective coverage and ability to maintain focus while preserving cognitive flexibility. 2. Depressive Disorder - Patient presents with flat affect, anhedonia, low energy, and sense of life passing by in context of chronic stress adaptation over more than 10 years. Symptoms distinct from major depressive disorder, characterized by emotional flatness and survival mode rather than typical depressive cognitions. Previous positive response to SSRI (Lexapro 5mg) supports pharmacologic intervention. 3. Burnout - Chronic occupational stress with adaptation of nervous system to prolonged fight-or-flight state. Patient describes baseline as survival mode with loss of ability to experience joy or stress response despite ongoing stressors.
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		effects or worsening mood. - Plan for ongoing medication management and therapy coordination once counseling established. Informed consent, including benefits and risks, was given by a medical decision maker for the patient. Items discussed include, but are not limited to the following: common side effects, alternatives, including no treatment and/or no pharmacotherapy. Risks, benefits, and alternatives of procedures, labs, and any other diagnostics performed today were reviewed with the patient prior to being completed.	
03/23/2026	Other	Psychopharmacology: - Continue sertraline at 25 mg once daily (full tablet daily in morning) for anxiety and depression. Patient responding well with excellent tolerability. Will reassess at 4-5 week mark to evaluate full therapeutic effect. - Discontinue Concerta. Current stimulant not providing adequate ADHD symptom control and causing fatigue, likely due to overmedication in context of improved anxiety baseline or suboptimal medication choice. - Plan to start Vyvanse 30 mg daily in 1-1.5 weeks after sertraline has additional time to stabilize (targeting 4-5 week point on sertraline). Vyvanse previously provided good coverage and focus before anxiety treatment. Starting at lower end of previous dose range (30-40 mg). Goal is to find lowest effective dose. Concerta can serve as backup if Vyvanse causes reactivation of anxiety symptoms. - Educated patient on rationale for medication timing - allowing sertraline to reach fuller therapeutic effect before reintroducing potentially activating stimulant. Referrals: - Referral to GPS Psychology and Anxiety Clinic sent for comprehensive psychological evaluation (including autism spectrum disorder assessment) and EMDR therapy. - Patient instructed to call GPS Psychology directly at contact information provided via text to expedite scheduling rather than waiting for clinic to call. - Will provide patient with alternative therapist referral list via patient portal due to mixed feedback regarding current GPS	1. Generalized Anxiety Disorder - Responding well to sertraline with significant early improvement in anxiety symptoms, decreased reactivity, and improved stress management. Reports 20-25% improvement within first 2-3 weeks, indicating good medication match. 2. Attention-Deficit/Hyperactivity Disorder, combined type - Current stimulant (Concerta) providing inadequate symptom control since reintroduction. Likely either overmedicated in context of improved anxiety baseline, undermedicated, or on

		counselor. List includes therapists experienced in ADHD, trauma-focused therapy, and EMDR, with location information. Patient can review biographies and select best fit. - Educated patient on using Psychology Today website to filter for insurance, specialty areas, and treatment modalities if additional resources needed. Follow-Up: - Follow up in 4-5 weeks for medication monitoring. Will reassess sertraline response at 4-5 week mark and evaluate Vyvanse tolerability and effectiveness after patient has been on it for approximately 1 month. - Patient to contact office sooner if concerns arise with medication transition. Informed consent, including benefits and risks, was given by a medical decision maker for the patient. Items discussed include, but are not limited to the following: common side effects, alternatives, including no treatment and/or no pharmacotherapy. Risks, benefits, and alternatives of procedures, labs, and any other diagnostics performed today were reviewed with the patient prior to being completed.	suboptimal medication. Previously responded well to Vyvanse prior to anxiety treatment. 3. Autism Spectrum Disorder (rule out)- Under consideration, referral placed for comprehensive evaluation.
		Psychopharmacology: - Continue sertraline at current dose for generalized anxiety disorder - excellent response with no side effects, optimal symptom control. Advised to please seek immediate medical attention if you have depression-like thoughts of suicide, new or increased anxiety, emotional instability, panic attacks, or mood changes. Pt verbalized understanding. - Restart Vyvanse 30 mg daily for ADHD mgmt - Vyvanse can be decreased to 20 mg if patient experiences increased anxiety, irritability, or other side effects after 1 week trial - Patient to call office if dose adjustment needed before next scheduled visit rather than waiting full month if medication is not tolerated - Educated patient that stimulants can worsen anxiety but Vyvanse has smoother delivery profile and sertraline is on board to help manage anxiety - Discussed that underdosing stimulants can also cause side	

04/27/2026	Other	effects, which is why starting at 30 mg rather than 20 mg - Monitoring for stimulant side effects, anxiety exacerbation, effectiveness for ADHD symptoms, and duration of coverage (approximately 10 hours expected) IDPMP reviewed when considering controlled substance medication. VS reviewed and are WNL. Urine drug screen will be utilized when necessary. ADD Controlled substance agreement form signed. Informed patient to take medication as prescribed. Stop taking the stimulant medication if experiencing mood instability, heart palpitations, chest pain, shortness of breath or any other cardiac symptom and seek immediate medical attention. Discussed risks vs benefits of treatment with medications today as well as possible adverse effects. Patient verbalized understanding today. Psychotherapy: -* Patient has first therapy appointment scheduled at Monarch on Thursday - Encouraged patient to assess fit with therapist and not feel obligated to continue if not good match - Therapy focus will include identity exploration, adjustment to improved mental health, processing of previous impairment and masking behaviors Laboratory/Diagnostic Testing: - Repeat screening questionnaires at next visit to assess symptom changes from baseline after starting sertraline Referrals: - Place referral to Northwest for comprehensive autism spectrum disorder evaluation and other potential diagnoses due to previous referral to GPS being unsuccessful after 3 months without response - Patient may also attempt in-person visit to GPS given lack of response to phone calls and referral Education: - Discussed that anxiety and ADHD can coexist and treating anxiety does not eliminate ADHD symptoms - Reviewed that patient's chronic anxiety was impairing quality of life and masking some ADHD symptoms - Educated that sertraline provides 24-hour coverage and requires consistent daily dosing	1. Generalized Anxiety Disorder - Excellent response to sertraline with resolution of chronic excessive worry, overplanning, fight-or-flight baseline, and anticipatory anxiety. Patient now recognizes previous anxiety symptoms that were not identified as pathological. 2. Attention-Deficit/Hyperactivity Disorder, predominantly inattentive type - Symptoms of inattention, impulsivity, and executive dysfunction persist but are more manageable with anxiety treatment. Focus difficulties and task completion challenges remain present though improved from baseline. 3. Autism Spectrum Disorder (provisional, evaluation pending) - Clinical presentation consistent with ASD including social communication difficulties, concrete and literal thinking, eye contact difficulties, masking behaviors, childhood special interests, and challenges with social reciprocity. Comprehensive neuropsychological evaluation recommended for diagnostic clarification.
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BERTLER, Rachel **DOB:** 11/03/1988 (37 yo F) **Acc No.** 28340

- Reviewed that manageable anxiety is normal and expected - goal is not complete elimination but rather preventing anxiety from being overwhelming and impairing

Follow-Up:

- Follow up in one month after to assess Vyvanse tolerability and effectiveness or sooner if needed

- Will reassess ADHD symptoms, anxiety symptoms, medication tolerability, and therapy engagement at next visit

Informed consent, including benefits and risks, was given by a medical decision maker for the patient. Items discussed include, but are not limited to the following: common side effects, alternatives, including no treatment and/or no pharmacotherapy.

Risks, benefits, and alternatives of procedures, labs, and any other diagnostics performed today were reviewed with the patient prior to being completed.

Plan Of Treatment

Next Appt

Details
Provider Name:Dina Mattingly, 06/08/2026 02:40:00 PM, 1526 W USTICK RD, STE 110, MERIDIAN, ID, 83646-7741, 208-247-0142

Goals Section

No Information

Health Concerns

No Information

Insurance Providers

Payer Name	Payer Address	Payer Phone	Subscriber Number	Group Number	Insured Name	Patient Relationship to Insured	Coverage Start Date	Coverage End Date
Aetna	PO BOX 981106 El Paso, TX 79998	888-632-3862	IDW277115898	19247014001	Bertler, Rachel	Self - patient is the insured		

Progress Notes

Examination

Date	Category	Field	Details	Notes	Category Notes
09/24/2025	Psychiatry	Appearance:	well-groomed, well-nourished		
		Psychomotor activity:	within normal range		
		Abnormal body movements:	none		
		Attention:	good		
		Affect / mood:	appropriate, full range euthymic		
		Speech / language:	appropriate pitch/modulation, clear and coherent, normal rate, volume, and articulation (RVR)		
		Thought process:	intact		
		Thought content:	appropriate		

Examination

Date	Category	Field	Details	Notes	Category Notes
					Appearance: Well-groomed, appropriate, appears stated age.
					Behavior: Cooperative, no abnormal movements.
					Speech: Normal rate, volume, and clarity.
					Mood: Improved, describes feeling calm and experiencing more joy.
					Affect: Appropriate and congruent.
					Thought Process: Logical and goal-directed.
					Thought Content: No SI/HI, no delusions.
					Reflective about identity and adjustment to
	Mini Mental Status Exam				

04/27/2026

improved mental health.
Perception: No hallucinations.
Cognition: Alert, oriented, memory intact.
Insight/Judgment: Excellent - demonstrates strong insight into anxiety symptoms, ADHD symptoms, and need for continued treatment.
Attention/Concentration: Adequate during visit.
Impulse Control: Good.

General Examination	General appearance:	alert, pleasant, well-nourished and in no acute distress
	Head:	normocephalic, atraumatic
	Heart:	regular rate and rhythm without murmurs, gallops, clicks or rubs
	Lungs:	clear to auscultation bilaterally, with good air movement and no rales, rhonchi or wheezes
	Neurologic:	alert and oriented
	Skin:	skin is warm and dry, with no rashes, good skin turgor and normal hair distribution
	Psych:	alert and oriented x 3

Examination

Date	Category	Field	Details	Notes	Category Notes
					Appearance: Well-groomed, appropriate, appears stated age. Behavior: Cooperative, no abnormal movements. Speech: Normal rate, volume, and clarity. Mood: "Flat." Affect: Constricted and congruent with

02/23/2026	Mini Mental Status Exam		depressed mood. Thought Process: Logical and goal-directed. Thought Content: No SI/HI, no delusions. Reports anhedonia and feeling life is passing by. Perception: No hallucinations. Cognition: Alert, oriented, memory intact. Insight/Judgment: Good - demonstrates awareness of chronic stress impact and seeks appropriate treatment. Attention/Concentration: Improved on current ADHD medication. Impulse Control: Good.		
	General Examination	General appearance: Head: Heart: Lungs: Neurologic: Skin: Psych:	alert, pleasant, well-nourished and in no acute distress normocephalic, atraumatic regular rate and rhythm without murmurs, gallops, clicks or rubs clear to auscultation bilaterally, with good air movement and no rales, rhonchi or wheezes alert and oriented skin is warm and dry, with no rashes alert and oriented x 3, cognitive function intact, cooperative with exam, maintains good eye contact, with good judgement and insight, normal affect / mood, with no auditory or visual hallucinations, speech is clear and coherent, thought process is logical and goal directed without suicidal ideation or delusions		

Examination

Date	Category	Field	Details	Notes	Category Notes
		Appearance:	well-groomed, well-nourished, appears stated age		
		Attitude:	cooperative		

11/17/2025

Psychiatry

Psychomotor activity: within normal range

Abnormal body movements: none

Attention: good

Degree of awareness of surroundings: within normal limits

Orientation: awake, alert and oriented x 3

Affect / mood: appropriate, full range

Speech / language: clear and coherent, normal rate, volume, and articulation (RVR), proper grammar used, appropriate pitch/modulation

Insight: good

Judgement: good

Thought process: intact

Thought content: appropriate

Perceptual disorders: no perceptual disorder noted

Aggression: not observed

Anger control: good

Suicidal ideation: none

Homicidal ideation: none

Impulse control: good

Delusions: no

Hallucinations: no

General Examination

General appearance: alert, pleasant, well-nourished and in no acute distress

Head: normocephalic, atraumatic

Heart: regular rate and rhythm without murmurs, gallops, clicks or rubs

Lungs: clear to auscultation bilaterally, with good air movement and no rales, rhonchi or wheezes

Neurologic: alert and oriented

Skin: skin is warm and dry, with no rashes

Psych: alert and oriented x 3, cognitive function intact, cooperative with exam, maintains good eye contact, with good judgement and insight, normal affect / mood, with no auditory or visual hallucinations, speech is clear and coherent, thought process is logical and goal directed without suicidal ideation or delusions

Examination

Date	Category	Field	Details	Notes	Category Notes
01/26/2026	Psychiatry	Appearance:	well-groomed, well-nourished, appears stated age		
		Attitude:	cooperative		
		Psychomotor activity:	within normal range		
		Abnormal body movements:	none		
		Attention:	good		
		Degree of awareness of surroundings:	within normal limits		
		Orientation:	awake, alert and oriented x 3		
		Affect / mood:	appropriate, full range		
		Speech / language:	clear and coherent, normal rate, volume, and articulation (RVR), proper grammar used, appropriate pitch/modulation		
		Insight:	good		
		Judgement:	good		
		Thought process:	intact		
		Thought content:	appropriate		
		Perceptual disorders:	no perceptual disorder noted		
		Aggression:	not observed		
		Anger control:	good		
		Suicidal ideation:	none		
		Homicidal ideation:	none		
		Impulse control:	good		
		Delusions:	no		
		Hallucinations:	no		
	General Examination	General appearance:	alert, pleasant, well-nourished and in no acute distress		
		Head:	normocephalic, atraumatic		
		Heart:	regular rate and rhythm without murmurs, gallops, clicks or rubs		
		Lungs:	clear to auscultation bilaterally, with good air movement and no rales, rhonchi or wheezes		
		Neurologic:	alert and oriented		
		Skin:	skin is warm and dry, with no rashes		

BERTLER, Rachel **DOB:** 11/03/1988 (37 yo F) **Acc No.** 28340

Psych: alert and oriented x 3, cognitive function intact, cooperative with exam, maintains good eye contact, with good judgement and insight, normal affect / mood, with no auditory or visual hallucinations, speech is clear and coherent, thought process is logical and goal directed without suicidal ideation or delusions

Examination

Date	Category	Field	Details	Notes	Category Notes
		Appearance:	well-groomed, well-nourished, appears stated age		
		Attitude:	cooperative		
		Psychomotor activity:	within normal range		
		Abnormal body movements:	none		
		Attention:	good		
		Degree of awareness of surroundings:	within normal limits		
		Orientation:	awake, alert and oriented x 3		
		Affect / mood:	appropriate, full range		
		Speech / language:	clear and coherent, normal rate, volume, and articulation (RVR), proper grammar used, appropriate pitch/modulation		
		Insight:	good		
	Psychiatry	Judgement:	good		
		Thought process:	intact		
		Thought content:	appropriate		
		Perceptual disorders:	no perceptual disorder noted		
		Aggression:	not observed		
		Anger control:	good		
		Suicidal ideation:	none		
		Homicidal ideation:	none		
		Impulse control:	good		
		Delusions:	no		
		Hallucinations:	no		
		General appearance:	alert, pleasant, well-nourished and in no acute distress		Diagnostics:
		Head:	normocephalic, atraumatic		Qb Interpretation:

10/20/2025

General
Examination

Heart:	regular rate and rhythm without murmurs, gallops, clicks or rubs	Patient is presenting with atypical symptoms of activity that are present throughout the entire test. The two most sensitive parameters for ADHD, Distance, and Area are (4.3) and (4.1) respectively, and show significant difficulty controlling hyperactivity. This patient's reaction time variation and omission errors, the two most sensitive parameters on QbTest 12-60 for difficulties with attention are (1.1) and (1.6), which is considered atypical compared to the age and gender-adjusted normative group and shows difficulty sustaining attention to the task. The patient's symptoms also increase over time into the test which is a pattern commonly seen in patients diagnosed with
Lungs:	clear to auscultation bilaterally, with good air movement and no rales, rhonchi or wheezes	
Neurologic:	alert and oriented	
Skin:	skin is warm and dry, with no rashes	
Psych:	alert and oriented x 3, cognitive function intact, cooperative with exam, maintains good eye contact, with good judgement and insight, normal affect / mood, with no auditory or visual hallucinations, speech is clear and coherent, thought process is logical and goal directed without suicidal ideation or delusions	

ADHD.Commission
Errors are (1.7)
which is atypical
and shows difficulty
controlling
impulsivity
compared to the
normative group.
This patient's
cardinal parameters,
which are weighted
summary scores that
summarize the
patients 3 core
symptom areas are
as follows:
QbActivity (3.8),
QbAttention (1.6),
and QbImpulsivity
(1.4) with
QbActivity and
QbAttention
indicating atypical
performances in
those symptom
areas.

Examination

Date	Category	Field	Details	Notes	Category Notes
		Appearance:	well-groomed, well-nourished, appears stated age		
		Attitude:	cooperative		
		Psychomotor activity:	within normal range		
		Abnormal body movements:	none		
		Attention:	good		
		Degree of awareness of surroundings:	within normal limits		
		Orientation:	awake, alert and oriented x 3		

12/15/2025	Psychiatry	Affect / mood:	appropriate, full range
		Speech / language:	clear and coherent, normal rate, volume, and articulation (RVR), proper grammar used, appropriate pitch/modulation
		Insight:	good
		Judgement:	good
		Thought process:	intact
		Thought content:	appropriate
		Perceptual disorders:	no perceptual disorder noted
		Aggression:	not observed
		Anger control:	good
		Suicidal ideation:	none
	General Examination	Homicidal ideation:	none
		Impulse control:	good
		Delusions:	no
		Hallucinations:	no
		General appearance:	alert, pleasant, well-nourished and in no acute distress
		Head:	normocephalic, atraumatic
		Heart:	regular rate and rhythm without murmurs, gallops, clicks or rubs
		Lungs:	clear to auscultation bilaterally, with good air movement and no rales, rhonchi or wheezes
		Neurologic:	alert and oriented
		Skin:	skin is warm and dry, with no rashes
		Psych:	alert and oriented x 3, cognitive function intact, cooperative with exam, maintains good eye contact, with good judgement and insight, normal affect / mood, with no auditory or visual hallucinations, speech is clear and coherent, thought process is logical and goal directed without suicidal ideation or delusions

Examination

Date	Category	Field	Details	Notes	Category Notes
					Appearance: Well-groomed, appropriate, appears stated age. Behavior: Cooperative, no abnormal

03/23/2026	Mini Mental Status Exam	movements. Speech: Normal rate, volume, and clarity. Mood: Improved. Affect: Appropriate and congruent. Thought Process: Logical and goal- directed. Thought Content: No SI/HI, no delusions. Perception: No hallucinations. Cognition: Alert, oriented, memory intact. Insight/Judgment: Good. Attention/Concentration: Adequate. Impulse Control: Good.						
	General Examination	General appearance:	alert, pleasant, well-nourished and in no acute distress					
		Head:	normocephalic, atraumatic					
		Neurologic:	alert and oriented					
			alert and oriented x 3, cognitive function intact, cooperative with exam, maintains good eye contact, with good judgement and insight, normal affect / mood, with no auditory or visual hallucinations, speech is clear and coherent, thought process is logical and goal directed without suicidal ideation or delusions					
		Psych:						

History and Physical Notes

HPI (History of Present Illness)

Date	Category	c/o	Denies	Symptom	Duration	Details	Notes	Category Notes
				Symptoms started		during childhood Struggled with reading and math as child. difficulty sustaining attention in tasks, difficulty completing tasks pt will start projects at work and at		

Attention deficit / hyperactivity disorder	Abnormal behaviors include	home will not finish them. difficulty organizing tasks delayed reading comprehension as a child. Would have re-read passages at times. Endorses impulsivity with spending, avoiding unpleasant or difficult tasks
	Symptoms include	being loud and intrusive, talking excessively, difficulty awaiting turn
Depression screening	PHQ-9	Little interest or pleasure in doing things: More than half the days
		Feeling down, depressed, or hopeless: Several days
		Trouble falling or staying asleep, or sleeping too much: Several days
		Feeling tired or having little energy: Nearly every day
		Poor appetite or overeating: Nearly every day
		Feeling bad about yourself or that you are a failure, or have let yourself or your family down: Several days
		Trouble concentrating on things, such as reading the newspaper or watching television: Nearly every

day

Moving or speaking so slowly that other people could have noticed; or the opposite, being so fidgety or restless that you have been moving around a lot more than usual: Not at all

Thoughts that you would be better off dead or of hurting yourself in some way: Not at all

Total Score: 14

Interpretation:
Moderate
Depression

Feeling nervous, anxious, or on edge: Several days

Not being able to stop or control worrying: Several days

Worrying too much about different things: Several days

Trouble relaxing: Several days

Being so restless that it is hard to sit still: Not at all

Becoming easily annoyed or irritable: Nearly every

day

Feeling afraid
as if something
awful might
happen:
Several days

Total GAD-7
Score: 8

If you checked
any problems,
how difficult
have they made
it for you to do
your work, take
care of things
at home, or get
along with
other people?:
Extremely
difficult

Interpretation
of Total: (5 to
9) Mild

How often do
you have
trouble
wrapping up
the final details
of a project,
once the
challenging
parts have been
done?: Very
Often.

How often do
you have
difficulty
getting things
in order when
you have to do
a task that
requires
organization?:
Sometimes.

How often do
you have
problems
remembering
appointments
or obligations?:
Often.

When you have

a task that
requires a lot of
thought, how
often do you
avoid or delay
getting
started?: Very
Often.

How often do
you fidget or
squirm with
your hands or
feet when you
have to sit
down for a long
time?:
Sometimes

How often do
you feel overly
active and
compelled to
do things, like
you were
driven by a
motor?: Often.

How often do
you make
careless
mistakes when
you have to
work on a
boring or
difficult
project?: Very
Often.

How often do
you have
difficulty
keeping your
attention when
you are doing
boring or
repetitive
work?: Very
Often.

How often do
you have
difficulty
concentrating
on what people
say to you,
even when they
are speaking to
you directly?:

Adult ADHD
Self-Report
Scale

Very Often.

How often do
you misplace or
have difficulty
finding things
at home or at
work?: Very
Often.

How often are
you distracted
by activity or
noise around
you?: Very
Often.

How often do
you leave your
seat in
meetings or
other situations
in which you
are expected to
remain seated?:
Rarely

How often do
you feel
restless or
fidgety?:
Sometimes

How often do
you have
difficulty
unwinding and
relaxing when
you have time
to yourself?:
Very Often.

How often do
you find
yourself talking
too much when
you are in
social
situations?:
Very Often.

When you're in
a conversation,
how often do
you find
yourself
finishing the
sentences of the
people you are

talking to,
before they can
finish them
themselves?:
Sometimes.

How often do
you have
difficulty
waiting your
turn in
situations when
turn taking is
required?: Very
Often.

How often do
you interrupt
others when
they are busy?:
Often.

Repeated,
disturbing, and
unwanted
memories of
the stressful
experience?:
Not at all

Repeated,
disturbing
dreams of the
stressful
experience?:
Not at all

Suddenly
feeling or
acting as if the
stressful
experience
were actually
happening
again (as if you
were actually
back there
reliving it)?:
Not at all

Feeling very
upset when
something
reminded you
of the stressful
experience?:
Not at all

Having strong
physical

Standardized
Measures

PCL-5

reactions when
something
reminded you
of the stressful
experience (for
example, heart
pounding,
trouble
breathing,
sweating)?: Not
at all

Avoiding
memories,
thoughts, or
feelings related
to the stressful
experience?:
Not at all

Avoiding
external
reminders of
the stressful
experience (for
example,
people, places,
conversations,
activities,
objects, or
situations)?:
Not at all

Trouble
remembering
important parts
of the stressful
experience?:
Not at all

Having strong
negative beliefs
about yourself,
other people, or
the world (for
example,
having
thoughts such
as: I am bad,
there is
something
seriously
wrong with me,
no one can be
trusted, the
world is
completely
dangerous)?:

Quite a bit (3)

Blaming
yourself or
someone else
for the stressful
experience or
what happened
after it?: Not at
all

Having strong
negative
feelings such as
fear, horror,
anger, guilt, or
shame?: Not at
all

Loss of interest
in activities that
you used to
enjoy?: A little
bit (1)

Feeling distant
or cut off from
other people?:
Not at all

Trouble
experiencing
positive
feelings (for
example, being
unable to feel
happiness or
have loving
feelings for
people close to
you)?: A little
bit (1)

Irritable
behavior, angry
outbursts, or
acting
aggressively?:
A little bit (1)

Taking too
many risks or
doing things
that could
cause you
harm?: Not at
all

Being
"superalert" or

watchful or on
guard?: A little
bit (1)

Feeling jumpy
or easily
startled?:
Moderately (2)

Having
difficulty
concentrating?:
Quite a bit (3)

Trouble falling
or staying
asleep?:
Moderately (2)

Total Score::
14

Has there ever
been a period
of time when
you were not
your usual self
and you felt so
good or so
hyper that other
people thought
you were not
your normal
self or you
were so hyper
that you got
into trouble?:
No

Has there ever
been a period
of time when
you were not
your usual self
and you were
so irritable that
you shouted at
people or
started fights or
arguments?: No

Has there ever
been a period
of time when
you were not
your usual self
and you felt
much more
self-confident
than usual?: No

Has there ever
been a period
of time when
you were not
your usual self
and you got
much less sleep
than usual and
found you
didn't really
miss it?: No

Has there ever
been a period
of time when
you were not
your usual self
and you were
much more
talkative or
spoke faster
than usual?: No

Has there ever
been a period
of time when
you were not
your usual self
and thoughts
raced through
your head or
you couldn't
slow your mind
down?: No

Has there ever
been a period
of time when
you were not
your usual self
and you were
so easily
distracted by
things around
you that you
had trouble
concentrating
or staying on
track?: No

Has there ever
been a period
of time when
you were not
your usual self
and you had
much more

energy than
usual?: No

Has there ever
been a period
of time when
you were not
your usual self
and you were
much more
active or did
many more
things than
usual?: No

Has there ever
been a period
of time when
you were not
your usual self
and you were
much more
social or
outgoing than
usual, for
example, you
telephoned
friends in the
middle of the
night?: No

Has there ever
been a period
of time when
you were not
your usual self
and you were
much more
interested in
sex than usual?:
No

Has there ever
been a period
of time when
you were not
your usual self
and you did
things that were
unusual for you
or that other
people might
have thought
were excessive,
foolish, or
risky?: Yes

Has there ever
been a period

of time when
you were not
your usual self
and spending
money got you
or your family
in trouble?: Yes

If you checked
YES to more
than one of the
above, have
several of these
ever happened
during the same
period of time?:
Yes

How much of a
problem did
any of these
cause you - like
being able to
work; having
family, money,
or legal
troubles;
getting into
arguments or
fights?: Minor
problem

Final::
Negative

Person(s)
Present:

Patient

Interim
History

Interim
Medical
Concerns:

Pt is concerned she has
adhd. Pt has been burned
out her whole. Was getting
worked up for medical
issues but everything came
back normal. It suggested
adhd may be a possibility
after reasearching pt
noticed she had many adhd
symptoms

HPI (History of Present Illness)

Date	Category	c/o	Denies	Symptom	Duration	Details	Notes	Category Notes
------	----------	-----	--------	---------	----------	---------	-------	-------------------

Little interest
or pleasure in
doing things:
Not at all

Depression
screening

PHQ-9

Feeling down,
depressed, or
hopeless: Not
at all

Trouble falling
or staying
asleep, or
sleeping too
much: Several
days

Feeling tired or
having little
energy: Not at
all

Poor appetite or
overeating: Not
at all

Feeling bad
about yourself
or that you are
a failure, or
have let
yourself or
your family
down: Not at
all

Trouble
concentrating
on things, such
as reading the
newspaper or
watching
television:
Several days

Moving or
speaking so
slowly that
other people
could have
noticed; or the
opposite, being
so fidgety or
restless that
you have been
moving around
a lot more than
usual: Not at all

Thoughts that
you would be
better off dead
or of hurting
yourself in
some way: Not

		at all
		Total Score: 2
		Interpretation:
		Minimal
		Depression
		Feeling
		nervous,
		anxious, or on
		edge: Several
		days
		Not being able
		to stop or
		control
		worrying: Not
		at all
		Worrying too
		much about
		different
		things: Not at
		all
		Trouble
		relaxing:
		Several days
		Being so
		restless that it
		is hard to sit
		still: Not at all
		Becoming
		easily annoyed
		or irritable: Not
		at all
		Feeling afraid
		as if something
		awful might
		happen:
		Several days
		Total GAD-7
		Score: 3
		If you checked
		any problems,
		how difficult
		have they made
		it for you to do
		your work, take
		care of things
		at home, or get
		along with
		other people?:
		Not difficult at
		all
Standardized Measures	GAD-7 (2018 Edition)	

Interpretation
of Total: (0 to
4) No Anxiety

HPI (History of Present Illness)

Date	Category	c/o	Denies	Symptom	Duration	Details	Notes	Category Notes
						Rachel B, a 34-year-old patient with a history of ADHD, presents for follow-up after discontinuing guanfacine due to adverse effects and experiencing ongoing challenges with ADHD management and work-related stress. She continues to take her Vyvanse at 40 mg once daily as prescribed and continues to tolerate this medication okay. The patient reports that she discontinued guanfacine before her last appointment after experiencing significant adverse effects including severe sleep disruption (only 3 hours of sleep per night), increased irritability, and worsening mental chatter - the opposite of the intended effects. She felt "not in a good place at all" while on the medication and experienced		

improvement within 4 days of discontinuation. Regarding her Vyvanse therapy, the patient has taken several days off recently due to having a cold and taking Sudafed, as well as during the guanfacine discontinuation period to "reset my central nervous system." She reports feeling uncertain about her current 40 mg dose, describing it as somewhere between optimal and suboptimal - the 30 mg dose provided good effects but insufficient duration, while 40 mg provides good coverage but sometimes feels "a little too high" with increased hyperfocus. She has noticed an increased resting heart rate over the past 2 weeks, though attributes this potentially to her recent illness. The patient describes significant struggles with time blindness and hyperfocus, particularly when working on personal projects like article writing. She reports spending 8 hours on tasks with

disproportionately small
output, getting caught in
cycles of editing and
elaborating that extend
project timelines
significantly. She
distinguishes this from
work-related tasks,
where she does not
experience the same
issues with real
deadlines, but notes that
her intense focus on
work consumes all her
energy, leaving none for
other activities. Since
receiving her ADHD
diagnosis, the patient
reports increased
awareness of her
limitations, which has
both positive and
negative aspects. While
she feels validated and
grateful for
understanding her
condition, she now
experiences more
anxiety about her
capabilities and
struggles to trust her
brain. She questions
whether her ideas and
projects are sustainable
or represent "dopamine
chases" that she will
abandon. This has
affected her confidence
in pursuing personal

goals like writing a book about navigating unstable work environments and starting a consulting business. The patient continues to experience work-related stress and has been reflecting on her career struggles, which she now recognizes as likely ADHD-related. She describes difficulty fitting into workplace environments and consuming excessive energy to maintain professional performance, leaving no capacity for personal interests or activities. She reports ongoing challenges with executive functioning, particularly with household tasks like dishes, despite having adequate time to complete them. The patient expresses feeling isolated and continues to struggle with consistency in daily activities. The patient has been considering the possibility of autism spectrum disorder, particularly given her

experiences with
hyperfixation,
difficulties with social
cues throughout her life,
challenges with sarcasm
and literal thinking, and
the significant effort
required for
interpersonal
communication. She
reports having to "coach
herself" on social
interactions due to
frequent
misunderstandings and
describes feeling like
she has to "put in so
much work just to
function around people."

Medical History -

ADHD- recently dx -

Performance anxiety,

notably during

competitive dog sports

and college exams -

Sleep difficulties - Low

ferritin levels on recent

blood work - Low

vitamin D levels on

recent blood work Social

History - Occupation:

Director of a project

management office -

Hobbies and interests:

Participates in

competitive dog sports -

Stress levels and coping

mechanisms: Reports

performance anxiety

related to competitive
activities, causing
physical symptoms
including nausea and
diarrhea; has history of
exam anxiety dating
back to college -

Exercise: Currently
walking dogs daily and
cycling 3-5 times per
week; reports not being
able to maintain
consistent exercise
routine for

approximately 10 years
prior to current period -

Substance Use:

Reintroduced caffeine
during second week of
current medication

regimen - Occupation:

Reports work-related
stress; participated in a
podcast Rachel Bertler
returns for 1-month

follow-up after
transitioning from

Vyvanse to Concerta for
ADHD management.

Reports preference for
Concerta but

experiencing significant
depressive symptoms
related to chronic stress
and burnout. Current

Symptoms: Patient
reports persistent
burnout despite ADHD
medication

improvement. Describes
over a decade of chronic
stress leading to altered
baseline functioning.

Experienced 2-week
depressive episode
approximately 1 week
ago characterized by
feeling life is "passing
by" and being in
"survival mode."

Reports everything feels
flat and hard to do.

Notes lack of joy but
distinguishes this from
typical clinical
depression, stating "I
don't feel like I've lost
joy. Like I feel like I
never had it to begin
with." Describes no
longer feeling stressed
or anxious at work
despite stressful job
environment - just
"living" and "surviving."

Impact on functioning
includes difficulty
engaging in activities
despite desire to
participate, limited
energy for weekend
activities, and feeling
unable to thrive despite
being in "prime" years at
age 37. Mood

Symptoms: Reports flat
affect and anhedonia but
denies feelings of failure

or typical depressive cognitions. Describes emotional flatness as longstanding rather than acute change. Notes occasional moments of energy (such as cleaning kitchen before bed) but must "take advantage" of these opportunities. Denies current anxiety or fight-or-flight response at work. No manic or hypomanic symptoms reported.

Attention/Concentration: ADHD symptoms improved on Concerta. Reports ability to focus while maintaining flexibility to redirect attention when needed, which is improvement over Vyvanse. On Vyvanse 30mg had inadequate coverage; 40mg caused excessive narrowing of focus with difficulty disengaging. Current Concerta dose provides good coverage throughout day. Able to context-switch as required by work environment.

Medications: Currently taking Concerta with good adherence and effectiveness. Reports

preferring Concerta over
Vyvanse due to better
balance of focus and
cognitive flexibility.
Medication helps with
executive function and
allows focus despite
feeling "like shit." Notes
ADHD medication
brings functioning above
current depressed
baseline, making
residual symptoms more
apparent. Denies end-of-
day crash or feeling
poorly when medication
wears off. Previous trial
of guanfacine resulted in
poor response. Previous
trial of Lexapro 5mg
was effective; increase
to 10mg in 2020 caused
intolerable side effects
including weight gain
and significantly
decreased libido. Recent
Stressors: Identifies
chronic occupational
stress as ongoing issue.
Works in environment
requiring frequent
context-switching.
Considering leaving
current job as believes it
perpetuates fight-or-
flight response.
Describes baseline
nervous system state as
adapted to chronic stress

02/23/2026 Interim
History

over more than 10 years,
comparing to type 2
diabetes model where
body stops responding
appropriately to repeated
hormone exposure.
Reports researching
peer-reviewed articles
on chronic stress
adaptation and its impact
on brain chemistry.
Therapy/Treatment
Adherence: Completed
release of information
form for referrals.
Awaiting ASD
evaluation at GPS
Psychology and Anxiety
Clinic after initial
referral location did not
accept insurance.
Interested in pursuing
EMDR therapy after
reading about its use for
chronic stress and
trauma. Identifies as
hyper-independent,
which previous therapist
noted as clinical term
associated with
childhood self-reliance.
Recognizes pattern of
having to "survive"
rather than thrive.
Engages in regular
exercise via Peloton,
which helps manage
ADHD symptoms. This
note was created with

the assistance of AI.

Rachel Bertler returns

for 1-month follow-up

after transitioning from

Vyvanse to Concerta for

ADHD management.

Reports preference for

Concerta but

experiencing significant

depressive symptoms

related to chronic stress

and burnout. Current

Symptoms: Patient

reports persistent

burnout despite ADHD

medication

improvement. Describes

over a decade of chronic

stress leading to altered

baseline functioning.

Experienced 2-week

depressive episode

approximately 1 week

ago characterized by

feeling life is "passing

by" and being in

"survival mode."

Reports everything feels

flat and hard to do.

Notes lack of joy but

distinguishes this from

typical clinical

depression, stating "I

don't feel like I've lost

joy. Like I feel like I

never had it to begin

with." Describes no

longer feeling stressed

or anxious at work

despite stressful job
environment - just
"living" and "surviving."
Impact on functioning
includes difficulty
engaging in activities
despite desire to
participate, limited
energy for weekend
activities, and feeling
unable to thrive despite
being in "prime" years at
age 37. Mood
Symptoms: Reports flat
affect and anhedonia but
denies feelings of failure
or typical depressive
cognitions. Describes
emotional flatness as
longstanding rather than
acute change. Notes
occasional moments of
energy (such as cleaning
kitchen before bed) but
must "take advantage"
of these opportunities.
Denies current anxiety
or fight-or-flight
response at work. No
manic or hypomanic
symptoms reported.
Attention/Concentration:
ADHD symptoms
improved on Concerta.
Reports ability to focus
while maintaining
flexibility to redirect
attention when needed,
which is improvement

over Vyvanse. On
Vyvanse 30mg had
inadequate coverage;
40mg caused excessive
narrowing of focus with
difficulty disengaging.
Current Concerta dose
provides good coverage
throughout day. Able to
context-switch as
required by work
environment.
Medications: Currently
taking Concerta with
good adherence and
effectiveness. Reports
preferring Concerta over
Vyvanse due to better
balance of focus and
cognitive flexibility.
Medication helps with
executive function and
allows focus despite
feeling "like shit." Notes
ADHD medication
brings functioning above
current depressed
baseline, making
residual symptoms more
apparent. Denies end-of-
day crash or feeling
poorly when medication
wears off. Previous trial
of guanfacine resulted in
poor response. Previous
trial of Lexapro 5mg
was effective; increase
to 10mg in 2020 caused
intolerable side effects

including weight gain
and significantly
decreased libido. Recent
Stressors: Identifies
chronic occupational
stress as ongoing issue.
Works in environment
requiring frequent
context-switching.
Considering leaving
current job as believes it
perpetuates fight-or-
flight response.
Describes baseline
nervous system state as
adapted to chronic stress
over more than 10 years,
comparing to type 2
diabetes model where
body stops responding
appropriately to repeated
hormone exposure.
Reports researching
peer-reviewed articles
on chronic stress
adaptation and its impact
on brain chemistry.
Therapy/Treatment
Adherence: Completed
release of information
form for referrals.
Awaiting ASD
evaluation at GPS
Psychology and Anxiety
Clinic after initial
referral location did not
accept insurance.
Interested in pursuing
EMDR therapy after

reading about its use for
chronic stress and
trauma. Identifies as
hyper-independent,
which previous therapist
noted as clinical term
associated with
childhood self-reliance.

Recognizes pattern of
having to "survive"
rather than thrive.

Engages in regular
exercise via Peloton,
which helps manage
ADHD symptoms. This
note was created with
the assistance of AI.

Rachel Bertler returns
for telehealth follow-up
after initiating sertraline
for anxiety. Reports
excellent tolerance and
early positive response
to medication. Current

Symptoms: Patient

reports feeling
significantly better
overall, describing
sensation of feeling
"lighter" and less
reactive to stressors.

Work environment
remains stressful but
able to manage more
effectively. Better able
to discern between
controllable and
uncontrollable factors
and let go of things

outside her control.

Anxiety Symptoms:

Marked improvement in
anxiety symptoms.

Reports decreased
reactivity and feeling
less need to respond to
everything. Situational
stressors at work
unchanged but
experiencing improved
coping. Noticed 20-25%
improvement between
weeks 2-3 of treatment.

Medications: Started
sertraline approximately
3 weeks ago on
February 28. Titrated
from half tablet daily for
1 week to full tablet
daily. Transition well-
tolerated with only 1 day
of headache and poor
sleep on the day of dose
increase, which resolved
quickly. Currently
taking full tablet daily in
the morning with 100%
adherence. To minimize
activation risk during
initiation, eliminated
caffeine, temporarily
held stimulant for first
week, and stopped B
vitamin supplements,
then gradually
reintroduced these.

Restarted Concerta
approximately 2 weeks

ago (around March 8)

after 1 week on

sertraline. Since

restarting, reports

stimulant feels

ineffective. Experienced

significant fatigue on

one occasion, feeling

able to nap within 3

hours of taking dose.

Previously on Vyvanse

with good coverage and

focus but discontinued

due to excessive

activation in context of

untreated anxiety.

Attention/Concentration:

Current stimulant

medication not

providing adequate

symptom control.

ADHD symptoms

persist despite

medication adherence.

Recent Stressors:

Ongoing work-related

stress, though improved

ability to manage

emotional response to

stressors.

Therapy/Treatment

Adherence: Referral sent

to GPS Psychology and

Anxiety Clinic for

comprehensive

evaluation (including

assessment for autism

spectrum disorder) and

EMDR therapy. Has not

yet contacted clinic to
schedule. Medical
History -ADHD-
recently dx -
Performance anxiety,
notably during
competitive dog sports
and college exams -
Sleep difficulties - Low
ferritin levels on recent
blood work - Low
vitamin D levels on
recent blood work Social
History - Occupation:
Director of a project
management office -
Hobbies and interests:
Participates in
competitive dog sports -
Stress levels and coping
mechanisms: Reports
performance anxiety
related to competitive
activities, causing
physical symptoms
including nausea and
diarrhea; has history of
exam anxiety dating
back to college -
Exercise: Currently
walking dogs daily and
cycling 3-5 times per
week; reports not being
able to maintain
consistent exercise
routine for
approximately 10 years
prior to current period -
Occupation: Reports

work-related stress;
participated in a podcast
This note was created
with the assistance of
AI.

HPI (History of Present Illness)

Date	Category	c/o	Denies	Symptom	Duration	Details	Notes	Category Notes
						Rachel, a director of a project management office, presents for follow-up to review QB testing results, having previously seen Kirk -NP-C for an initial ADHD evaluation. She reports chronic fatigue and difficulty with executive functioning that has significantly impacted her work and personal life. The patient describes longstanding patterns of needing to work "under the gun" and creating pressure for		

herself to
complete tasks
effectively. She
reports
particular
difficulty with
closing out
projects, noting
this occurs both
at work and in
personal tasks
around the
house, where
"the last little
things of
wrapping it up
is the hardest
thing." She has
observed
similar patterns
in employees
who report to
her who have
ADHD
diagnoses.
Rachel reports
increased
awareness of
fidgeting
behaviors since
beginning the
evaluation
process. She
describes subtle
movements
during
meetings,
including
shifting from

side to side in
her chair, which
she had not
previously
noticed until
she began
paying attention
to these
behaviors. She
notes that when
working
remotely and
observing
colleagues on
camera, she
realized "they're
not moving at
all, they're like
statues" and
questioned
whether her
level of
movement was
normal. Sleep
difficulties have
been
problematic,
described as
"pretty bad."
She was
recently
prescribed
mirtazapine by
a virtual
provider
through
Brightside
specifically to
help with sleep

while awaiting

ADHD

evaluation

results. The

patient

describes a

history of

impulsive

behaviors,

particularly in

her early

twenties,

including poor

financial

management.

She reports

asking herself

"why am I even

acting like this?

Why am I doing

these things?"

She had

developed a

narrative that

she was "just

bad at money"

and "never

learned the

tools," though

she now

questions

whether this

may have been

related to

undiagnosed

ADHD. Reports

symptoms have

been present

since childhood.

Rachel reports
chronic fatigue
spanning
approximately
10 years, during
which she
sought medical
evaluation
including blood
work. Despite
being told she
was "the picture
of health," she
consistently felt
unhealthy.
Recent lab work
showed low
ferritin and low
vitamin D
levels, though
not at
significant
levels according
to her provider.
She describes
feeling tired and
less motivated
to do things.
She reports
developing
binge-watching
habits as a
coping
mechanism,
describing
television as
"something I
can do that
doesn't require

my mind to
really focus."
She notes
difficulty with
shows that
require
significant
involvement,
sometimes
needing to
rewind content.
The patient
describes
accidentally
intermittent
fasting daily,
attributing this
to difficulty
with "the task of
preparing food
and then sitting
down and
eating it." She
reports having
laundry that
"will pile up for
months
sometimes" and
searching for
clean clothing.
Rachel reports
severe
performance
anxiety,
particularly
noted during
competitive dog
sports. She
describes a

recent
competition
where anxiety
caused physical
symptoms
including
diarrhea and
nausea, stating
she was
"making myself
sick." She
recalls similar
anxiety during
college exams.
She has never
tried medication
for this anxiety,
though recalls a
provider
mentioning beta
blockers in
college, which
she was hesitant
to try at the
time. Medical
History -
Performance
anxiety, notably
during
competitive dog
sports and
college exams -
Sleep
difficulties -
Low ferritin
levels on recent
blood work -
Low vitamin D
levels on recent

blood work

Medications

and

Supplements -

Mirtazapine 7.5

mg for sleep -

Sometimes

takes just half

the dose -

Supplements

tried to manage

what patient

didn't know was

ADHD Social

History -

Occupation:

Director of a

project

management

office - Hobbies

and interests:

Participates in

competitive dog

sports - Stress

levels and

coping

mechanisms:

Reports

performance

anxiety related

to competitive

activities,

causing

physical

symptoms

including

nausea and

diarrhea; has

history of exam

History of
10/20/2025 Presenting
Problem

anxiety dating
back to college
Rachel B., a
patient with
ADHD, returns
for a one-month
follow-up visit
after starting
Vyvanse 30mg,
reporting
significant
improvement in
executive
functioning and
quality of life.
She describes
feeling "hugely
validated" as
she realizes the
extent of her
previous
struggles with
burnout,
inability to
complete
household tasks
like dishes and
laundry,
inconsistent dog
walking,
grocery
shopping
difficulties, and
exercise
inconsistency.
Since starting
Vyvanse, the
patient reports
dramatic

improvements
in her ability to
form and
maintain habits.
She has been
consistently
walking her
dogs every
single day and
cycling 3-5
times per week,
noting she
hasn't been able
to maintain
consistent
exercise
routines in
approximately
10 years. She
purchased a
Peloton for her
birthday and
has been using
it regularly,
expressing
previous
concerns that it
would become
another unused
item but finding
herself
motivated and
getting stronger.
During the first
week on
medication, she
experienced
significant
focus

improvement
with only minor
sleep challenges
on two nights,
specifically
difficulty falling
asleep quickly
while
maintaining
ability to stay
asleep. Her
thoughts
became more
organized, and
she noticed a
marked
improvement in
meeting
presentations
where she
typically over-
prepared due to
tendency to
"talk in circles."
She was able to
give an
organized,
flowing update
without full
preparation,
which made her
realize "this is
how normal
people are
supposed to
function." In
week two, she
was able to
reintroduce her

morning coffee

after

understanding

the medication's

effects.

However, she

experienced

increased

impulsivity with

micro-behaviors

such as

consistently

checking her

phone and

getting up

without

purpose. She

began noticing

the medication's

effects

diminishing

around 5 PM,

which became

problematic for

after-work

committee

meetings. By

week three, the

medication's

effectiveness

appeared to

decrease

further, with

effects wearing

off around 3

PM. She

experienced

increased

hyper-focus

episodes where

she would

complete tasks

until finished

regardless of

time

management.

Meal

management

difficulties

returned during

this period, with

instances of not

eating until 2

PM despite

previously

maintaining

regular

breakfast

routines.

Currently in

week four, she

reports feeling

fully adjusted to

the medication

with continued

success in

maintaining

new habits like

dog walking

and cycling,

noting she

typically would

have given up

by this point.

However, she

continues to

experience

shortened

duration of
medication
effectiveness
and ongoing
difficulties with
household tasks
including
laundry, dishes,
and grocery
shopping
despite being
"on" the
medication. The
patient remains
on mirtazapine,
which she
reduced from
7.5mg nightly
to half a pill
every other
night due to
increased
snacking
behaviors and
morning
grogginess. She
reports the
appetite effects
of mirtazapine
and Vyvanse
seemed to
counteract each
other initially,
but the
mirtazapine
caused
problematic
over-snacking
patterns. She

experiences
minimal side
effects from
Vyvanse,
reporting only
slight dry
mouth that
resolves with
adequate
hydration. She
has
implemented
several
organizational
systems
including
writing "L" and
"R" on contact
lenses, adding a
to-do list widget
to her phone,
and recognizing
that typical
ADHD
management
strategies like
"making it
easy" do not
work for her
personal needs.
She expresses
interest in
optimizing her
daily schedule
to capitalize on
periods of
clarity and
focus while
working around

ADHD

challenges.

Medical History

-ADHD-

recently dx -

Performance

anxiety, notably

during

competitive dog

sports and

college exams -

Sleep

difficulties -

Low ferritin

levels on recent

blood work -

Low vitamin D

levels on recent

blood work

Social History -

Occupation:

Director of a

project

management

office - Hobbies

and interests:

Participates in

competitive dog

sports - Stress

levels and

coping

mechanisms:

Reports

performance

anxiety related

to competitive

activities,

causing

physical

symptoms
including
nausea and
diarrhea; has
history of exam
anxiety dating
back to college
- Exercise:
Currently
walking dogs
daily and
cycling 3-5
times per week;
reports not
being able to
maintain
consistent
exercise routine
for
approximately
10 years prior to
current period -
Substance Use:
Reintroduced
caffeine during
second week of
current
medication
regimen -
Occupation:
Reports work-
related stress;
participated in a
podcast

Consultation Request Notes

Open Referral(s)			
Referral	Referring	Referred Provider	Notes

Date	Provider		
04/27/2026	Mattingly, Dina	Northwest Neurobehavioral,	Comprehensive evaluation for ASD and other potential diagnoses

Addressed Referral(s)

Reason	Can you please place a referral for CBT that accepts her insurance. thanks! Monarch Behavioral Health
Diagnosis 1	Attention-deficit hyperactivity disorder, combined type (F90.2)
Referral Organization	ADD Clinic Of Idaho
Referral Status	Addressed
Referral Date	01/26/2026
Referring Provider First Name	Dina
Referring Provider Last Name	Mattingly
Referring Provider Speciality	Nurse Practitioner
Referred Provider	Monarch Behavioral Health
Referred Provider Specialty	Mental health counseling
General Notes	Haas, Brandie 04/13/2026 03:52:09 PM MDT >Referral faxed to Monarch Behavioral Health
Referral Priority	Routine

Reason	Can you please place a referral to GPS psychology and anxiety clinic for eval for possible ASD. thanks
Diagnosis 1	Attention-deficit hyperactivity disorder, combined type (F90.2)
Diagnosis 2	Performance anxiety (F41.8)
Diagnosis 3	Current episode of major depressive disorder without prior episode, unspecified depression episode severity (F32.9)
Diagnosis 4	Work stress (Z56.6)
Referral Organization	ADD Clinic Of Idaho
Referral Status	Addressed
Referral Date	01/26/2026
Referring Provider First Name	Dina
Referring Provider Last Name	Mattingly
Referring Provider Speciality	Nurse Practitioner
Referred Provider	Gps Psychology And Anxiety
Referred Provider Specialty	Psychologist
General Notes	Haas, Brandie 02/18/2026 12:14:42 PM MST >Sent ROI to be able to send referral to GPS, Haas, Brandie 02/23/2026 10:23:59 AM MST >Referral faxed.
Referral Priority	Routine

Physical Findings

No Information

Care team

Personal relationship	Rachel Bertler
Contact info	2099 N WATERBROOK PL STAR, ID 83669-6127, US Tel: +1(310)-750-5113